

Mental Health & Psychiatry

Diagnostic Criteria, Management Principles & Crisis Protocols

Document ID: MCA-006 | Version 1.0 | Category: Psychiatry & Mental Health | MediCare AI Solutions

1. Depressive Disorders

1.1 Major Depressive Disorder (MDD)

MDD is characterised by persistent low mood and/or anhedonia (loss of interest/pleasure) causing significant functional impairment. It is one of the leading causes of disability globally. Diagnosis requires DSM-5/ICD-11 criteria.

DSM-5 Diagnostic Criteria (Abbreviated)

Five or more of the following symptoms present for ≥ 2 weeks, representing a change from baseline; at least one must be depressed mood or anhedonia:

- Depressed mood most of the day, nearly every day
- Markedly diminished interest or pleasure in almost all activities (anhedonia)
- Significant weight change ($\geq 5\%$ in a month) or appetite disturbance
- Insomnia or hypersomnia nearly every day
- Psychomotor agitation or retardation (observable by others)
- Fatigue or loss of energy nearly every day
- Feelings of worthlessness or excessive/inappropriate guilt
- Diminished ability to think, concentrate, or make decisions
- Recurrent thoughts of death or suicidal ideation with/without plan

■ **SAFETY PRIORITY:** Always assess for suicidal ideation in patients with depression. Ask directly: 'Are you having thoughts of harming yourself or ending your life?' A safety plan should be formulated for at-risk patients.

1.2 Treatment of Depression

Severity	First-Line Treatment	Additional Options
Mild	Psychoeducation, lifestyle measures, watchful waiting	Low-intensity CBT, guided self-help, exercise
Moderate	Antidepressant (SSRI) + structured psychotherapy (CBT/IPT)	Combined pharmacotherapy + therapy preferred

Severity	First-Line Treatment	Additional Options
Severe	Antidepressant (higher dose) + high-intensity CBT	Consider psychiatric referral; augmentation agents
Treatment-Re sistant	Trial 2nd agent; augmentation (lithium, atypical AP)	ECT considered for severe/refractory or psychotic depression

2. Anxiety Disorders

2.1 Generalised Anxiety Disorder (GAD)

GAD is characterised by excessive, uncontrollable worry about multiple life domains persisting for ≥ 6 months, with associated somatic and psychological symptoms causing functional impairment.

- Excessive anxiety and worry about multiple topics (work, health, finances, family)
- Difficulty controlling worry
- At least 3 of: restlessness/feeling on edge, fatigue, concentration difficulty, irritability, muscle tension, sleep disturbance
- Causes significant distress or functional impairment
- Not explained by substances, medical condition, or another mental disorder

2.2 Panic Disorder

Panic disorder is defined by recurrent unexpected panic attacks with persistent concern about future attacks or maladaptive behavioural changes. Attacks peak within minutes and resolve within 20–30 minutes.

Panic Attack Symptoms (≥ 4 required for full attack):

- Palpitations, pounding heart, or accelerated heart rate
- Sweating, trembling/shaking
- Shortness of breath or feeling of being smothered
- Chest pain or discomfort
- Nausea or abdominal distress
- Dizziness, lightheadedness, or faintness
- Derealization or depersonalisation
- Fear of losing control or 'going crazy'
- Fear of dying, paresthesias, chills/hot flushes

■ Medical causes must be excluded before a primary anxiety disorder diagnosis: cardiac arrhythmia, hyperthyroidism, phaeochromocytoma, hypoglycaemia, substance intoxication/withdrawal.

3. Psychotic Disorders

3.1 Schizophrenia

Schizophrenia is a chronic, severe mental disorder affecting cognition, emotion, and behaviour. It typically emerges in late adolescence to early adulthood and follows a relapsing-remitting or chronic course.

Symptom Domain	Examples
Positive Symptoms	Hallucinations (auditory most common), delusions (persecutory, grandiose, referential), disorganised thought/speech, disorganised or catatonic behaviour
Negative Symptoms	Blunted/flat affect, alogia (poverty of speech), avolition, anhedonia, asociality — often more disabling long-term
Cognitive Symptoms	Impaired working memory, attention, executive function, processing speed — often present before positive symptoms

■ **First-episode psychosis:** Early intervention is critical. All patients require urgent psychiatric assessment, antipsychotic medication, and psychosocial support. Delay in treatment is associated with poorer outcomes.

4. Mental Health Crisis Protocols

4.1 Suicide Risk Assessment

Structured risk assessment tools (e.g., Columbia Suicide Severity Rating Scale — C-SSRS) assist in stratifying risk, but clinical judgement remains paramount. Key risk factors include:

- **HIGH RISK FACTORS:** Previous attempt (strongest predictor), specific plan, access to means, hopelessness, psychosis with command hallucinations, recent significant loss
- **MODERATE RISK:** Current ideation without plan, substance use, social isolation, chronic pain, male gender (higher lethality methods)
- **PROTECTIVE FACTORS:** Strong social support, religious/moral objections, children at home, reasons for living, engaged in treatment, future-orientation

■ **CRISIS RESPONSE:** For active suicidal ideation with plan/intent — do not leave the patient alone, remove access to means, activate emergency services or ensure immediate inpatient psychiatric assessment. Document everything.

4.2 De-escalation Principles

- Approach calmly, maintain respectful, non-threatening body language and low tone

- Active listening — validate emotions without agreeing with delusions
- Offer choices where safe to do so; reduce environmental stimulation
- Avoid arguments, commands, or physical threats unless immediate safety risk
- Involve trusted individuals if present and appropriate
- Pharmacological de-escalation (lorazepam ± haloperidol) for severe agitation — specialist guidance

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